

Should Dad switch from Cymbalta (duloxetine) to Lexapro (escitalopram) ahead of the June 8 ketamine restart?

RECOMMENDATION

The switch direction is defensible given Dad's decades-long prior-responder history on Lexapro. The load-bearing question is the timing relative to the June 8 ketamine restart. Two pieces of the medical case for the switch are real: Dad functioned well on Lexapro at 10–20 mg for many years before the recent severe phase, and there's some chance the SNRI noradrenergic component of duloxetine is contributing to current anxiety/agitation. Two pieces of the case against are also real: Lexapro previously failed at high doses (30–50 mg) when the depression escalated in 9–10/2025, and a 3-week cross-titration before the ketamine restart layers two pharmacological transitions in the same window — duloxetine withdrawal-related anxiety and Lexapro start activation risk would both land before any benefit window (4–8 weeks for SSRI anxiolytic effect). The recommended ask of Dr. Rubin tomorrow is sequencing: run the first 2–3 ketamine sessions on the existing duloxetine background, then revisit the switch with response data in hand. Julane's preferred sequencing pointed the same direction. If pre-ketamine timing isn't winnable, the next-best medical structure is a gradual cross-titration (≥1 week of overlap at duloxetine 30 mg, then 20 mg, then off), capping Lexapro at 5–10 mg (the historical effective range, not the previously-failed high-dose range), and defining explicit symptoms that would trigger reversal (akathisia, persistent agitation, new motor symptom in the first 2 weeks).

WHY THIS MATTERS FOR DAD SPECIFICALLY

Dr. Chilton has pushed the A2 ketamine start to June 8 specifically to allow time for a Cymbalta → Lexapro switch first, on the stated reasoning that she considers starting ketamine on Cymbalta riskier than on Lexapro. Dr. Rubin appears poised to operationalize the switch. Dad's documented February 2026 IV ketamine response was on a duloxetine + brexpiprazole background — duloxetine is the established responder context. Dad also has decades of pre-severe-phase functioning on Lexapro at 10–20 mg; lit review #11 (4/20/2026) noted "poor response to high-dose escitalopram (30–50 mg)" when the depression escalated, which is what prompted the duloxetine switch in 9/2025. The medical question is whether re-trial of Lexapro at lower doses works at the current severity. The timing question is whether the 3-week cross-titration window is the right place to do this experiment, given it overlaps with the ketamine restart.

THE EVIDENCE

What supports the switch (especially with the right sequencing)	What argues against pre-ketamine timing — in Dad's specific profile
Dad has decades of strong functioning on Lexapro at 10–20 mg. This is real prior-responder data — meaningful N=1 evidence that aggregate trial estimates don't capture. Patient-level prior response is one of the strongest predictors of re-response when an agent is re-trialed after a gap.	Lexapro previously failed at high doses (30–50 mg) when the depression escalated. Lit review #11 documented "poor response to high-dose escitalopram (30–50 mg)" — exactly the context (severe depression) that's relevant now. Re-trial at low doses might work, but the responder evidence is from a less-severe Dad on lower doses, not the current severity profile. The high-dose failure is the closest comparator.
SNRI noradrenergic activity may contribute to current anxiety/agitation. Duloxetine's NE reuptake inhibition	Documented Feb 2026 ketamine response was on duloxetine. Removing duloxetine removes the established pharmacological

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can produce or exacerbate anxiety in some patients. Dad's prominent anxiety component (including the 5/18 session distress and the recent body-shaking episodes) is at least partly compatible with this mechanism. Pure SSRI may be more comfortable.	context for the only intervention that has demonstrably worked at this severity. Del Casale 2025 (n=55,480) found SNRI + esketamine outperformed SSRI + esketamine on mortality (5.3% vs 9.1%), hospitalization, and relapse. Both lines argue for preserving the SNRI background at least until the ketamine response is re-established.
Lexapro is well-tolerated in elderly and broadly safe. A long-standing, well-characterized agent. If the switch doesn't work, it's reversible. The downside isn't catastrophic — Dad has functioned on this drug class for decades. Cardiac profile is favorable for his bradycardia + LAFB.	Duloxetine discontinuation syndrome is non-trivial. A 2025 JAMA Psychiatry meta-analysis: 1.61 additional symptoms on DESS scale at week 1, peaks 36–96 hours, can persist 2–6 weeks. Acute anxiety, paresthesias, "brain zaps," nausea, agitation, insomnia. In an elderly patient with prominent anxiety, the withdrawal symptoms would be indistinguishable from worsening depression and would confound the ketamine response read at the most important moment.
NPI + Julane weekly + ketamine carry the main therapeutic load anyway. Even if the Lexapro switch is neutral (~40–50% probability) or modestly negative (~25–35%), the family's energy is better spent on the non-pharmacologic side than fighting the AD direction. The switch is not the load-bearing decision.	Lexapro start activation in older adults overlaps with the ketamine restart. Per the OE response, prior history of drug-induced akathisia/EPS is a documented predisposing factor for SSRI activation (Lane 1998). The AMSP surveillance program identified escitalopram as the most frequently implicated SSRI in EPS case reports. Dad's activation-intolerance pattern across multiple agents puts him in the elevated-risk category. The activation window is 1–4 weeks — landing right on the ketamine restart.
—	SSRI anxiolytic effect takes 4–8 weeks. Even if Lexapro is the right destination, the benefit window doesn't land until well after the ketamine sessions. The "less acutely anxious before the next session" goal Chilton stated isn't actually achievable by the switch in the available time.
—	Julane's preferred sequencing. Run the first 1–2 ketamine sessions on the existing stack, observe response, revisit any antidepressant switch with that data in hand. The most conservative version of the same proposal — doesn't introduce two pharmacological changes in the same window; preserves the duloxetine background the documented response was on; gives the most clinical information per unit risk.

HONEST PROBABILITY READ

- **~20–30% the switch helps** — real prior-responder status finds Dad again on Lexapro at lower doses (10–20 mg).
- **~40–50% the switch is neutral** — Dad's depression at this severity doesn't respond meaningfully to either AD as the load-bearing intervention; ketamine and NPIs carry the work.
- **~25–35% the switch makes the June window harder** — duloxetine withdrawal anxiety + Lexapro start activation overlap with the ketamine restart, with loss of duloxetine background that backed the documented response.

WHAT EVERYONE AGREES ON

- **Lexapro is on the table — the disagreement is about timing.** Dad's long historical responder status is real; the debate is whether to do the switch before or after the ketamine sessions.
- **Julane's sequencing (first sessions on duloxetine, then revisit) is the conservative form of the same proposal.** Not a fundamentally different plan — just a different order.
- **The documented Feb 2026 responder background is duloxetine.** Preserving it through the first sessions costs little and protects the highest-value variable on the table.
- **The bupropion-augmentation option is a third path** if "do more than ketamine" pressure remains — see [handout #21](#). Adds rather than switches; mechanistically distinct from prior intolerances; OPTIMUM data favors augmentation over switching.
- **NPI work doesn't depend on the AD decision.** Caffeine, bright light, exercise, alcohol abstinence, digital CBT-I, weekly Julane — all proceed regardless.

RECOMMENDED NEXT STEPS

1. **Tomorrow's Rubin call (5/20, 11 AM):** open with the sequencing request — "Could we run the first 2–3 ketamine sessions on duloxetine to preserve the Feb 2026 responder background, and revisit the switch with response data at the mid-June check-in? Julane suggested the same sequencing." This is the strongest medical position and concedes the direction (Lexapro is on the table) while protecting the most valuable variable.
2. **If sequencing isn't winnable,** accept the switch but ask for:
 - **Gradual cross-titration** — duloxetine 30 mg for ≥ 1 week of overlap, then 20 mg for another week, then off; minimizes the withdrawal-anxiety window.
 - **Lexapro dose cap at 5–10 mg.** The historical effective range. Don't push to the high doses (30–50 mg) where Dad previously did poorly.
 - **Explicit reversal triggers defined upfront:** akathisia, persistent agitation, new motor symptom in the first 2 weeks, any fall, worsening depression in the first 4 weeks.
 - **Plan to assess at ~3 weeks vs ~8 weeks** on whether the switch is helping; budget for the long-arm benefit window not landing until well after the ketamine sessions.
3. **Bupropion augmentation as a third path** if "do more" pressure remains: instead of switching from duloxetine to Lexapro, consider adding bupropion XL 150 mg to duloxetine. Per [handout #21](#), this has stronger OPTIMUM evidence than switching and preserves the documented responder background. Fall risk is the load-bearing caveat.
4. **Decline the doxepin and trazodone removal pieces of the proposal regardless of the AD decision.** Lemborexant can be added (Julane-endorsed) as a sleep-maintenance addition without removing what's working. If Dad wants to come off doxepin specifically, the doxepin → lemborexant swap is seamless per [handout #10](#); trazodone PRN stays.
5. **NPI work proceeds in parallel** regardless of the AD decision. See [handout #19](#).
6. **Weekly Julane sessions through the gap to June 8** are the strongest non-pharmacologic lever on the "felt alone" dimension that distinguished 5/18 from February.

BOTTOM LINE *The switch direction is defensible — Dad's decades of good functioning on Lexapro at 10–20 mg is real prior-responder data. The OE evidence against the switch is real but is mostly about the previously-failed-high-dose context, the documented duloxetine + ketamine responder background, and the timing/window concerns. The recommended ask of Rubin tomorrow is sequencing rather than direction: first 2–3 ketamine sessions on duloxetine, revisit the switch with response data in hand. If sequencing isn't winnable, gradual cross-titration with a Lexapro dose cap at 5–10 mg and explicit reversal triggers is the next-best medical structure.*